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Clover Health

June 16, 2025

Dear Stephanie Carlton, Deputy Administrator of the Centers for Medicare & Medicaid Services and Dr. Thomas Keane, M.D., Assistant Secretary for Technology Policy, National Coordinator for Health Information Technology:

I write on behalf of Clover Health Investments, Corp. ("**Clover**"). We are a physician-enablement technology company committed to bringing access to great healthcare to everyone on Medicare. Clover has long advocated for more choice and competition in healthcare, as well as efforts to reduce costs for seniors, improve care quality, and to efficiently use taxpayer dollars. Our company offers Medicare Advantage ("**MA**") plans, with the majority of our members being enrolled in our wide network PPO plans. The following are the foundational principles of our plans:

- *Patient Choice and Affordability*: Our members choose their own providers through broad networks, and minimal in- v. out-of-network cost-sharing differences. We offer $0 to low monthly plan premiums and copays with a maximum out-of-pocket limit.

- *Health Technology for All*: We empower clinicians to detect and manage chronic conditions earlier by equipping them with longitudinal patient data and AI-generated insights at the point of care and in their work flows. We do so through our technology platform, Clover Assistant, for in-network, primary-care physicians in our company's MA plans, and through Counterpart Assistant for primary-care physicians in other plans. Clover Assistant and Counterpart Assistant provide the same platform (collectively here, "**Counterpart Assistant"**). Through Counterpart Assistant, we proudly provide the most sophisticated care-coordination tools (often available only to integrated, large health systems), to independent physicians groups in rural and underserved communities.

- *The Right Care, at the Right Time, for Our Most Vulnerable Members*: Through our team of nurses, doctors, and other healthcare professionals, we identify high-risk members, and give them preventative care directly in their homes. And our teams use our cutting-edge AI platform, Counterpart Assistant, to provide superior care.

We respond to CMS, ASTP, and ONC's request for information on the state of data interoperability and broader health technology infrastructure ("**RFI**") on behalf of both our

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Medicare Advantage plans, and our healthcare technology business, Counterpart Health, Inc. ("**Counterpart**"), which brings access to Counterpart Assistant to clinicians across the MA landscape, through relationships with health systems participating in value-based care programs, value-based care enablers, and other MA payors.

Clover provides proposals in the five following areas:

- improving access to valuable data across the healthcare ecosystem;
- reducing data duplication and strengthening data provenance;
- strengthening interoperability through TEFCA and policy alignment;
- empowering patients through improved access; and
- clarifying the role of authorized vendors under the Information Blocking Rule.

Below please find a brief overview of Clover's business and model of care, followed by our proposals in each area. We thank you in advance for your consideration of our proposals, and would be grateful for the opportunity to discuss data interoperability and health technology infrastructure issues with you further.

I. Clover's Model of Care & Technology Platform

Clover was founded in 2014 with the mission of improving every life. Today, we offer MA plans in New Jersey, Georgia, Pennsylvania, South Carolina, and Texas. As of 2025, our plans have over 100,000 members. What distinguishes Clover's MA plans from others is our care delivery model, described below:

A. Broad Networks and Affordability

We believe that patients are best off when they see a doctor of their own choosing—someone the patient trusts in good times and in bad to have their best interests in mind. So unlike many other payors that funnel members through their narrow provider networks (often filled with provider groups they have purchased over the years), we give our members broad networks and honor their choices. In most of our plans, we double down on that commitment to patient choice by minimizing any cost-sharing differential between in- and out-of-network providers. Nearly all of our members are in PPO plans with large provider networks and no referral needed. And we offer $0 to low monthly plan premiums and copays with a maximum out-of-pocket limit.

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B. Cutting-Edge Technology

Clover empowers clinicians to detect and manage chronic conditions earlier through its proprietary healthcare technology platform, Counterpart Assistant. Unlike many payors, whose technology focus is often on the "insurance" aspect of managed care—*e.g.*, using AI to more tightly manage claims processing and prior authorizations—our focus is on helping clinicians improve care quality.[1]

Counterpart Assistant does this by aggregating data from over 100 sources across the healthcare system. We use AI and our machine-learning models to curate patient-specific insights from the data based on clinical standards of care. And we deliver those insights to clinicians at the point of care. For example:

- If pharmacy records ingested by Clover reflect that a member has a prescription that is overdue for pickup, Counterpart Assistant can let the doctor know, so she can nudge the patient or find out if there is some socioeconomic or other deterrent she or Clover can help with.

- If Clover has no record of a member receiving a colorectal cancer screening in the timeline reflected in clinical guidelines, Counterpart Assistant can let the doctor know about the potential need and the pertinent guidelines. Within the tool, the doctor can prompt Clover to send a screening test directly to the patient's home.

- If lab records ingested by Clover reflect that a member has a GFR (a measure of kidney function) suggesting possible chronic kidney disease, Counterpart Assistant can let the doctor know so she can evaluate the patient for the condition. And if the doctor determines the patient has chronic kidney disease, Counterpart Assistant will surface potential treatment options for the doctor to consider, and the clinical guidelines supporting those options (*e.g.*, prescribing the patient a medication known as an ACE inhibitor).

The results speak for themselves: using Clover Assistant is correlated with better results, including the following:

- *Congestive Heart Failure (CHF)*: CHF patients who had relationships with primary-care providers using the Clover Assistant had a lower average number of all-cause hospitalizations (18% lower) and 30-day readmissions (25% lower) compared to patients whose provider did not have access to Clover Assistant. Clover Assistant helped physicians facilitate more

[1] And to be clear, we do *not* use AI for prior authorization. At Clover, any exploration of AI use in utilization management would be focused on providing *faster* "yeses." And any denial would be made by a qualified human decision maker after appropriate review.

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proactive and frequent longitudinal specialty care management for medically complex CHF patients who often face mobility challenges and reduced access to traditional office-based sites, including: (1) higher utilization of in-home care (22.8% versus 12.33% receiving at least 1 in-home visit); and (2) more outpatient cardiologist visits (1.1 visits for CA patients versus 0.8 visits for non-Clover Assistant patients).[2]

- *Diabetes*: Compared to those who did not have Clover Assistant visits with their primary-care providers, enrollees who had Clover Assistant-assisted provider visits: (1) primary-care practitioner use of Clover Assistant is correlated with earlier diagnosis (average HbA1c 6.65% for members having a Clover Assistant visit with a PCP and 6.91% for members not having such a visit); (2) were treated for diabetes an average of 36 months earlier; (3) were much more likely to obtain earlier treatment with oral diabetes medications and delay use of insulin at later stages of type 2 diabetes; and (4) had lower documented rates of hypoglycemia, a severe complication of diabetes, at 1,000 days (1.5% versus 2.4%).[3]

- *Chronic Kidney Disease (CKD)*: Primary-care physicians who used Clover Assistant were able to diagnose–earlier–CKD stage 3 and higher. Earlier diagnosis of CKD resulting from CA-assisted visits slowed the decline of kidney function significantly (GFR, or glomerular filtration rate, of about 4.6 mL/min per year versus 1.3 mL/min per year), helping to delay and prevent adverse and costly health outcomes from reduced kidney function, including high blood pressure, heart disease, stroke, dialysis dependence, and death.[4]

- *Medication Adherence*: Non-adherence to medications for diabetes, high blood pressure, and high cholesterol is common and associated with worse chronic disease control, increased disease-related complications, and higher healthcare costs. Clover Assistant visits alerted primary-care physicians to patients' medication non-adherence. Medication fills increased by 4.8% on the day enrollees had a CA-assisted visit with a primary-care provider and remained 2.6% higher 90 days after the visit.[5]

Outcomes like these–from earlier diagnosis and management of chronic conditions to better medication adherence–are what led Clover's PPO plans to a 4 Star overall rating for Star Ratings year

[2] See here for more details.
[3] See here for more details.
[4] See here for more details.
[5] See here for more details.

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2025, and to achieve 4.94 out of 5 Stars on HEDIS measures, the top-performing score on core HEDIS measures for Medicare Advantage plans nationwide.[6]

C. In-Home Care

Clover understands that for some members, getting to the doctor is a challenge, whether because of mobility problems, social reasons, or other constraints. Thus, for high-risk members, Clover offers to bring care directly to the home through both in-person and telehealth visits. Our in-home care teams, which include physicians and nurses employed by Clover, use Counterpart Assistant to bring AI-powered care-coordination to our most vulnerable members. And unlike some other programs, the purpose of these visits is not just to document conditions, but to provide impactful longitudinal care. Again, the results speak for themselves: members enrolled in Clover's in-home care program in 2023 saw a ~26% reduction in inpatient stays, compared to just a 2.7% reduction of eligible members who were not enrolled in the program.

II. Improving Access to Valuable Data Across the Healthcare Ecosystem

As highlighted in Questions PC-8 and PC-9 of this RFI, access to complete, high-quality health data is essential for effective care coordination, population health, and digital innovation. But some of the most valuable datasets remain difficult to access, standardize, or exchange across health systems.[7] We provide the following recommendations to improve access.

- *Appointment Scheduling*: Regarding RFI Questions PC-8, PC-12, and PR-4, appointment scheduling data plays a critical role in coordinating timely care and patient engagement. But without a national standard, scheduling data remains inconsistently structured across systems. We encourage development of a FHIR-based national standard for appointment scheduling to promote greater interoperability and alignment across platforms.

- *Expanding Access Beyond Blue Button*: In response to RFI Questions PC-9 and TD-2.a., we strongly urge expanding the data available through its APIs, such as the Blue Button 2.0 API,

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[6] 4.94 is the weighted average of the measure scores in the CMS Star Ratings for which the primary data source was HEDIS and for which Clover was eligible to be rated. The calculation uses the same weighting used by CMS for the measures in calculating the overall Star score. The analysis targets non-SNP (Special Needs Plan) HEDIS measures for plans with more than 2,000 contracted lives. Exclusions include measures like C06 (Medication Review) and C07 (Pain Assessment), which are only relevant to SNP plans. Not all contracts offer SNPs, therefore those measures were excluded to enhance the accuracy of comparison across contracts. For detailed methodology and the HEDIS performance of the broader industry visit, please see here.

[7] Centers for Medicare & Medicaid Services. *Request for Information; Health Technology Ecosystem.* Question PC-8. May 2024. Available at: https://www.federalregister.gov/documents/2025/05/16/2025-08701/request-for-information-health-technology-ecosystem

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to fully align with the comprehensive data requirements mandated for other payers under the Patient Access and Payer-to-Payer API rules. While Blue Button 2.0 has laid a strong foundation for patient-directed access to Medicare Fee-for-Service data, currently only claims data (for the most part) is available from CMS. That limitation significantly constrains robust, interoperable solutions essential for effective care management. CMS also holds a broader set of patient-related data, including social determinants of health, care gap status, and quality performance metrics, that is not currently accessible through authorized tools. Access to this clinical data would empower beneficiaries and their caregivers to meaningfully engage in their care and enhance providers' ability to identify care gaps and improve care coordination.

- *Imaging and Diagnostics*: In response to RFI Questions PC-8, PC-9 and TD-13, we highlight that imaging and diagnostic data are largely siloed across disperate imaging systems and often unavailable across care settings in real time. These gaps delay clinical decision-making and limit the ability of health-technology platforms to support informed care delivery. To address these challenges and unlock the full potential of electronic health information, we encourage considering policies that promote structured exchange of imaging data through national frameworks, including via TEFCA and FHIR-based APIs, to ensure this valuable data is accessible when it is needed.

A. Reducing Data Duplication and Strengthening Provenance

Addressing core themes raised in RFI Questions PC-11 (data accuracy), PR-7 (high quality and comprehensive data), TD-10 (effortless EHI access), and TD-13 (obstacles to full EHI), we note that data duplication has long been a challenge in healthcare, leading to bloated patient records and uncertainty about which version of a record is most reliable. Most recently, new duplication patterns have emerged as more organizations participate in national exchange frameworks. For example, after retrieving data via a network query, some organizations may include those previously obtained records in their own outbound responses, circulating the same information multiple times and compounding existing duplication risks.

While standards for data provenance exist, adoption and consistency remain limited. We encourage CMS to work with ONC to incorporate detailed requirements for provenance tagging and data reuse directly into the ONC Health IT Certification Program to ensure that exchanged information remains accurate, deduplicated, and clinically meaningful. For example, CMS and ONC could develop standards under which repeated patterns of data duplication would be considered a form of information blocking.

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B. Strengthening Interoperability through TEFCA and Policy Alignment

We strongly support efforts to advance TEFCA and believe additional steps could help ensure it fulfills its role as an equitable framework for health information exchange. Our recommendations for maximizing TEFCA's impact directly address inquiries raised in RFI Questions PC-10 (TEFCA's impact on patient access), PR-6 (TEFCA's role in provider access), and TD-6 (interoperability functions). To maximize TEFCA's impact, we recommend:

- *Broader Participation*: While TEFCA remains voluntary, please explore future pathways such as certification requirements or participation incentives that encourage adoption among certified EHR vendors, laboratories, and other critical data holders. For example, ambient-scribing vendors could contribute raw transcription–with the appropriate patient consent and other guardrails–to ensure accuracy of ambient-scribing and related AI products. One way to encourage such participation is to reduce or eliminate various regulatory or programmatic burdens for those who use TEFCA. We are happy to discuss various de-regulatory avenues in this regard.

- *Expanding Required Exchange Purposes*: We encourage expanding TEFCA-required use cases to more accurately reflect the full scope of permissible data exchange under HIPAA, particularly with respect to treatment and healthcare operations.[8] Under the current framework, QHINs and their participants are only required to respond to requests for "Treatment" and "Individual Access Services."[9] This limitation impedes the flow of health information crucial for modern healthcare delivery and forces reliance on less efficient non-standardized methods for data exchange. Accordingly, we propose expanding the mandated scope of "Treatment" to include critical care coordination and pre-encounter planning activities (e.g., pre-admission planning, proactive care management, medication adherence, care gap evaluation). Concurrently, we urge making "Healthcare Operations" a mandated use case to directly enable the robust data flow essential for value-based care, comprehensive population health, and quality improvement initiatives. Mandating this exchange is essential to achieve true nationwide interoperability, fostering an effective, patient-centered healthcare system.

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[8] 45 C.F.R. § 164.501 (defining "treatment" to include the provision, coordination, or management of healthcare and related services by one or more providers, including with third parties; defining "healthcare operations" to include quality assessment and improvement activities, case management, and care coordination).

[9] The Sequoia Project, *Standard Operating Procedures (SOP): Exchange Purposes (XPs)*, Version 3.0 (Aug. 6, 2024), available at https://rce.sequoiaproject.org/wp-content/uploads/2024/08/SOP-Exchange-Purposes_CA-v3_508.pdf

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- *Reciprocal Data Exchange Obligations*: Pertaining to RFI Questions PR-6, TD-6, TD-16, and VB-11, we propose that to maintain trust, ensure balanced participation, and realize nationwide interoperability, entities participating in TEFCA must be unequivocally required to timely respond to inbound data queries. Without clear and enforceable reciprocal obligations, the framework risks becoming a unilateral data flow, undermining care coordination and ultimately impeding patient outcomes. Timely data response, subject to robust privacy, security, and technical feasibility safeguards, is foundational for enabling critical workflows like clinical decision-making, care transitions, and population health management.

- *Historical Data Access*: Please enhance the Payer-to-Payer API (and Provider Access API) rules to mandate or incentivize impacted payers to proactively transfer a beneficiary's historical data (up to the five-year look-back) to their subsequent plan(s) upon disenrollment. The Interoperability and Prior Authorization Final Rule (CMS-0057-F) currently requires that "impacted payers" (including MA plans) make five (5) years of a member's historical data (e.g., claims, encounters, clinical data, etc.) available to their new plan, upon member consent, through a Payer-to-Payer API. But this requirement does not cover any plan(s) other than the one that the patient has just left for the new plan. Given that many patients switch plans several times within a five-year period, the existing requirement provides significantly less than five years of patient data to the new plan. We recommend closing this loophole and making clear that the five-year period extends to all plans that the beneficiary was a member of within the five years, and that all such plans must automatically transfer data to the patient-chosen plan upon appropriate notice.

- *Expansion of the TEFCA Manner Exception*: The current limitation of the TEFCA manner exception creates a significant disincentive for early adoption, directly addressing concerns about policy limitations and suggested changes to TEFCA as raised in RFI Questions PA-1 and PR-6. This exception currently requires both the actor and requestor to be connected to TEFCA for it to apply, meaning actors may be less likely to invest if they cannot use TEFCA as the default exchange mechanism for the majority of requests.[10] To further TEFCA's interoperability functions and consolidate redundant channels (as discussed in TD-6), and consistent with calls to revise exceptions to further EHI access (PR-12), we encourage expanding this exception. Allowing actors to route EHI requests through TEFCA even if the requester is not a TEFCA participant would reduce reliance on less standardized exchange methods, promote a unified technical framework, and substantially advance the overarching goal of nationwide interoperability.

- *Harmonize TEFCA and State Privacy Laws*: The complex landscape of state privacy laws, particularly for sensitive health information, remains one of the most significant hurdles to ensuring secure and compliant interoperability within TEFCA, directly addressing the policy

[10] 45 C.F.R. § 171.403.

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limitations and hindrances to participation raised in RFI Question PA-1.[11] There are presently no clear guidelines on how QHINs and their participants must navigate these restrictive state-specific requirements, which increases the risk of unintentional privacy violations, legal exposure, and loss of patient trust and thereby hinders TEFCA's broader adoption. To rectify these concerns and better balance patient privacy with data access as explored in Question PR-9, please consider issuing comprehensive, actionable guidance on harmonizing TEFCA operations with these varying state privacy laws, specifically detailing methodologies to identify applicable regulations, to implement robust data segmentation, and to honor the most stringent consent requirements across the network. Greater clarity will not only bolster data privacy and security, but it will also build the essential trust required to expand health data exchange nationwide.

C. Empowering Patients through Improved Access

Although patients are increasingly empowered to access their health information, many still face significant barriers related to authentication and digital access. Password-based systems often pose challenges for seniors or individuals without reliable digital access. To meaningfully address these challenges, as explored in RFI Questions PC-14, PR-9, PR-10, and TD-3, we strongly advocate requiring all Patient Access API requests to be satisfied with a NIST 800-63-3 IAL2 authenticated request. This would standardize digital identity credentials (such as those provided by CLEAR, Login.gov, or ID.me) for health access. This critical step, while maintaining appropriate privacy and security protections, would support more accessible and inclusive authentication methods, help extend the benefits of patient data access to populations often left behind by traditional credentialing systems, and further enhance patient-centered care.

D. Clarifying the Role of Authorized Vendors Under the Information Blocking Rule

Health-technology vendors like Counterpart play a critical role in enabling providers to deliver timely, informed treatment at the point of care. Acting under valid HIPAA-compliant business associate agreements, we facilitate the exchange and use of electronic health information to directly support provider-directed treatment, care coordination, and clinical quality. Unfortunately, we have encountered restrictions or fees imposed by data holders. These restrictions limit and delay access, even when the request is made on behalf of a covered entity for a permitted purpose.

Writing as a technology vendor directly experiencing these challenges, and in response to RFI Question TD-18 (examples of information blocking practices), these practices often manifest as unreasonable contractual barriers, excessive and non-standardized data access fees, technical

[11] *See, e.g.*, Cal. Code, HSC § 123110; N.Y. Pub. Health Law § 2782; Fla. Stat. § 381.004(3)(e) (imposing stricter standards for disclosure of certain types of sensitive health information such as HIV status, mental health, and reproductive care).

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impediments, or outright non-responsiveness to legitimate requests for EHI access on behalf of providers.

This interference with provider-directed data access severely compromises care delivery workflows and patient outcomes. It undermines the "without special effort" principle for EHI access (as emphasized in RFI Question TD-10) and inhibits market competition, a concern raised in Question PR-12.

We recommend clarifying that interference with provider-directed access, particularly through systematic practices that impede the usability of EHI, may still constitute information blocking, even if executed by a non-actor at the direction or allowance of an actor. Furthermore, in alignment with RFI Question TD-18 (policies to discourage information blocking), there should be a clear, efficient and well-publicized process for authorized vendors to raise and resolve such concerns. This would help ensure that data access is not improperly obstructed and preserve the integrity of the health-information ecosystem.

Conclusion

We are very excited to work with you on continuing to improve data interoperability in healthcare. We firmly believe that the dividends from this work will benefit stakeholders across the healthcare ecosystem, including, most importantly, the patients. We would appreciate the opportunity to further discuss data interoperability and health-technology infrastructure issues.

Best regards,

Signed by:
Will Chang
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Will Chang
Vice President and Head of Government Affairs